

ORTHOPEDIC SURGERY - OPERATIVE / CLINICAL NOTE

Patient Name: Stacy Truman

Date of Birth: 01/01/1970

Member ID: 123456

Insurance: BlueCross Premier PPO

Date of Service: December 5, 2024

Procedure: Total Knee Arthroplasty (TKA), Right Knee

CPT Code: 27447

Surgeon: Robert Hayes, MD - Orthopedic Surgery

Facility: Capitol Orthopedic Surgical Center

PREOPERATIVE DIAGNOSIS

- M17.11 Primary osteoarthritis of the right knee
- M25.561 Pain in right knee, chronic

POSTOPERATIVE DIAGNOSIS

- M17.11 Primary osteoarthritis of the right knee (confirmed intraoperatively; full-thickness articular cartilage loss of the medial compartment)

INDICATION / MEDICAL NECESSITY

Ms. Truman is a 54-year-old female with longstanding right knee pain secondary to severe primary osteoarthritis (M17.11). She has failed an extended course of conservative management over more than 12 months, including NSAIDs, a structured physical therapy program, activity modification, and two intra-articular corticosteroid injections, with only transient relief.

Weight-bearing radiographs of the right knee demonstrate severe tricompartmental degenerative change with bone-on-bone medial joint-space loss, marginal osteophyte formation, and subchondral sclerosis - consistent with end-stage osteoarthritis. She has progressive functional impairment affecting activities of daily living, ambulation, and stair use.

Given the radiographic severity of the osteoarthritis and the failure of conservative treatment, the patient is an appropriate candidate for total knee arthroplasty. Relevant comorbidities - type 2 diabetes mellitus (E11.9), atherosclerotic heart disease (I25.10), and essential hypertension (I10) - were medically optimized preoperatively and cleared for surgery.

DESCRIPTION OF PROCEDURE

Under general anesthesia, a standard medial parapatellar approach to the right knee was performed. Intraoperative findings confirmed severe osteoarthritis with full-thickness cartilage loss of the medial femoral condyle and tibial plateau, marginal osteophytes, and degenerative changes of the patellofemoral joint. Standard femoral and tibial resections were carried out and a cemented modular total knee prosthesis was implanted with good stability and range of motion. The patient tolerated the procedure well.

ASSESSMENT / PLAN

Right knee primary osteoarthritis (M17.11), status post total knee arthroplasty (CPT 27447). The procedure is medically necessary and directly supported by the documented diagnosis of end-stage osteoarthritis of the right knee. Routine post-operative care, DVT prophylaxis, and outpatient physical therapy arranged.

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